APIONA / CLINICAL RECORD CONCEPT

Hypermobility: Clinical Summary

Illustrative patient report • Medical history and physiotherapy context

DEMONSTRATION DOCUMENT — Fictional case for interface prototyping. Not a signed medical report, a clinical diagnosis, or instructions for treatment.

Patient & record context

Patient	Camille M. (fictional) • 28 years
Reason for visit	Midfoot sprain after a fall on stairs; emergency radiograph reported no fracture. Nine physiotherapy sessions prescribed.
History noted in source wireframes	Hypermobile Ehlers-Danlos syndrome (hEDS) listed as a documented diagnosis; general practitioner report attached in the fictional record.
Clinical status	This summary reproduces illustrative record content; no examination or independent diagnostic verification was performed.

Relevant documented / reported history

Joint history	Joint laxity and instability, recurrent sprains, proprioceptive difficulties; previous recurrent shoulder dislocations and left shoulder stabilization surgery (2021).
Additional conditions	Chronic migraines and frequent dizziness; cerebral cavernoma history; asthma. These are separate considerations and should not be automatically attributed to hEDS.
Current symptoms	Midfoot pain and swelling with limping; discomfort while walking, at night, and while seated. Right knee pain and right wrist discomfort also reported.
Patient priorities	Pain relief, improved sleep, clarity about expected recovery, and practical home exercises.

Immediate review considerations

• Confirm allergies and current medications, including names and doses; these fields are incomplete in the illustrative record.

• Review supplied source reports before relying on any diagnosis or prior test result.

• Assess current injury, functional limitations, balance and fall risk within the clinician’s scope; account for reported dizziness and neurological history.

• Avoid treating every reported symptom as a consequence of hypermobility; evaluate alternative or coexisting causes as clinically indicated.

Record provenance: adapted from the Apiona team’s fictional Camille M. wireframes (patient summary and initial consultation). No new patient facts have been invented.

APIONA / CLINICAL REFERENCE

Hypermobility: Assessment & Care Context

Educational reference to support interpretation of the sample patient report

Understanding hypermobility

Joint hypermobility describes joints that move beyond the usual range. It may be asymptomatic. When associated with symptoms or impairment, clinicians consider hypermobility spectrum disorders (HSD) and, where criteria are met, hypermobile Ehlers-Danlos syndrome (hEDS). These are clinical classifications, not conclusions that can be made from joint flexibility alone.

Diagnostic approach

History and examination. Document pain, instability, recurrent injuries, functional impact, family history, and systemic features; assess hypermobility with an appropriate examination, including the Beighton score when applicable.

hEDS criteria. Adult hEDS diagnosis uses established clinical criteria including generalized joint hypermobility, required additional features and exclusion of alternative causes.

HSD consideration. Symptomatic hypermobility not meeting hEDS criteria may be classified as HSD after appropriate clinical evaluation.

Testing. There is currently no laboratory or genetic test that confirms hEDS; investigations may be used to assess other conditions when indicated.

Common management principles

• Individualized physiotherapy emphasizing strength, controlled movement, balance and proprioception.

• Activity modification and pacing to reduce repetitive strain or end-range overextension.

• Functional goals aligned with the patient’s priorities, with reassessment of symptoms and participation.

• Multidisciplinary review when clinically indicated, particularly for complex or persistent symptoms.

Information to verify in an actual clinical record

Diagnostic evidence	Beighton assessment, diagnostic criteria review, clinician name, assessment date, and original signed report.
Medication & safety	Medication list and doses, allergies, relevant precautions, concurrent diagnoses.
Patient perspective	Patient-described symptoms, impact on daily life, goals, and questions for consultation.
Traceability	Source of each statement; distinguish patient report, clinician assessment, and imported document.

Clinical references

The Ehlers-Danlos Society — Diagnostic Criteria
GeneReviews — Hypermobile Ehlers-Danlos Syndrome
NHS — Joint Hypermobility Syndrome

Prepared 04 November 2026 • Illustrative educational material only. Clinical decisions require assessment by a qualified professional.